



# **Type 2 DM Updates Unity Health Care**

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# Outline

## **Part 1: General Guideline Revisions**

**\* Key : Those in Red – recommend to include in Algorithm**

## **Part 2: Modified Type 2 Algorithm**

# Part 1 :General ADA guideline revisions;

- Recommendations Adopted from: *Diabetes Care* 2026;49(Suppl. 1):S6–S12 Doi: <https://doi.org/10.2337/dc26-SREV>
- **New Section Endorsements:**
  - **CKD and Risk management** – endorsed by National Kidney Foundation
  - **Children and Adolescent section** - endorsed by International Society for Pediatric and Adolescent Diabetes
- Updated Recommendations
  - 1) **Improving Care and Promoting Health in Populations**
    - Emphasis on shared decision making on health outcome
    - Digital self management tools
    - Need for community health workers to support management of kidney risk factors( on top of DM and CVD rf.)
    - **Recommendation of CDCES and RDN's that specialize in older adults**

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## **2. Diagnosis and Classification of Diabetes**

- **Prompt evaluation of stage 3 (overt) type 1 diabetes**
- **monitoring of individuals with specific autoantibodies, such as IA-2(high risk of progression)**
- **Hyperglycemia management in Cancer treatment(including glucocorticoids, immune checkpoint inhibitors, and PI3K $\alpha$  inhibitors.)**
- **Refine Cystic fibrosis –related diabetes**
- **Gestational Diabetes screening protocols**

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### 3. Prevention or Delay of Diabetes and associated comorbidities

- Continuous glucose monitoring (CGM) for those with presymptomatic type 1 diabetes
- Prevention strategies for type 2 diabetes emphasize achieving at least a 5–7% weight loss through Mediterranean or low-carbohydrate eating patterns and technology-assisted programs.
- Metformin – prevent hyperglycemia in high-risk individuals treated with high-dose glucocorticoids or PI3K $\alpha$  inhibitors.

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## 4. Comprehensive Medical Evaluation and Assessment of Comorbidities

- Thorough assessment of glycemic status, current treatments, and **available support systems or care partners**
- **Older adults** - osteoporosis drug therapy based on specific T-scores and fracture risks.
- **Glucagon-like peptide 1 receptor agonists (GLP-1 Ras)** - for patients with type 2 diabetes and metabolic dysfunction–associated steatohepatitis (MASH), particularly those at high risk for liver fibrosis

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## 5. Facilitating Positive Health Behaviors and Well-being to Improve Health Outcomes

- Diabetes self-management education and support (**DSMES**)
- Safety assessments for religious fasting, tobacco and vaping cessation counseling, and annual screenings for anxiety and fear of hypoglycemia.
- Weight management - minimum of 5–7% weight loss while monitoring nutritional intake

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## 6. Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises

- Promotes the inclusion of oral glucose in first aid kits for public settings like schools and workplaces to treat hypoglycemia
- Glycemic goal-setting has been individualized to include **specific CGM metrics** based on a person's health status and treatment factors
- Expanded content on the outpatient management and prevention of DKA



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# 7. Diabetes Technology

- recommend initiating **CGM at diabetes onset** for **all individuals on insulin or other therapies that carry a risk of hypoglycemia**
- **Automated insulin delivery (AID) systems** – for Type 1 DM and Type 2 DM on multiple daily injections
- Updated CGM definitions and need to be included in schools and workplace.

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## 8. Obesity and Weight Management for the Prevention and Treatment of Diabetes

- **Annual obesity screening is now recommended using BMI** supplemented by additional adiposity measurements where feasible
- even a 5–7% weight loss improves glycemia
- minimizing the use of medications that promote weight gain
- GLP-1 RA-based therapies and metabolic surgery – Obesity rx. for type 1

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# 9. Pharmacologic Approaches to Glycemic Treatment

- Use of **dual GIP and GLP-1 Ra** - for patients with type 2 diabetes and symptomatic heart failure (HFpEF) or MASH.
- **AID Systems for all adults on insulin**
- Hyperglycemia management during cancer treatment and organ transplant
  - Immediate post op- Insulin
  - Long-term – Non-insulin Rx.

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# 10. Cardiovascular Disease and Risk Management

- Systolic blood pressure goal **<120 mmHg** should be encouraged for individuals with high cardiovascular or kidney risk.
- Maximum tolerated dose of ACEI/ARB for RX. of **HTN** in those with **severely increased albuminuria and/or estimated glomerular filtration rate (eGFR) <60 mL/min/1.73 m<sup>2</sup>**
- **GLP-1 RAs and dual GIP/GLP-1 Ras** – reduce **CVD** events

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# 11. Chronic Kidney Disease and Risk Management

- Focus on **SBP<120mmHG**
- **Simultaneous initiation of an SGLT2 inhibitor and a nonsteroidal mineralocorticoid receptor antagonist (nsMRA) for patients** with albumin-to-creatinine ratio  $\geq 100$  mg/g with eGFR 30–90 mL/1.73 m<sup>2</sup> on a renin-angiotensin-aldosterone system (RAS) inhibitor.
- safety information for kidney-protective medications in pregnant individuals and those on dialysis

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# 12. Retinopathy, Neuropathy, and Foot Care

- **Retinopathy** : effects of GLP-1 RA treatment on various eye conditions
- **Neuropathy**: emphasizes combination therapy for pain relief and align with foot care protocols
- **Foot care**: Use of technology (smart Mats) for early ulcer detection and the **WIFI**(Wound Ischemia foot Infection) staging system

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# 13. Older Adults

- **Adults on insulin – CGM**
- Treatment goal: **older adults of <130/80 mmHg** when it can be achieved safely and a more relaxed blood pressure goal (e.g., **<140/90 mmHg**) for people with poor health, limited life expectancy, or high risk for adverse effects of hypertensive therapy.
- **specific daily protein intake targets (0.8 g/kg) and exercise recommendations to maintain lean body mass during weight loss.**

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# 14. Children and Adolescents

- **emphasizing family-centered care and psychosocial support**
- **routine screening for social determinants of health and mental health issues like diabetes distress starting as early as age seven**
- **new pharmacotherapy data for GLP-1 RAs and SGLT2 inhibitors in pediatric populations**



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# **15. Management of Diabetes in Pregnancy**

## **Preconception care**

- **Emphasize achieving glucose goals and discontinuing GLP-1 RA or dual GIP/GLP-1 RA therapies before attempting to conceive.**
- **blood pressure threshold for initiating therapy in pregnancy is now set at 140/90 mmHg**
- **technology-related guidance for CGM and AID use in pregnancy**

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# 16. Diabetes Care in the Hospital

- perioperative period - a target A1C of <8% within three months of elective surgery and a blood glucose range of 100–180 mg/dL
- updated diagnostic criteria for hyperglycemic crises like DKA and HHS
- patients not being discharged home - evaluating the destination facility's capabilities for diabetes management

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# 17. Diabetes Advocacy

- NO revisions

# Edits to the Management Algorithm

## Adopted from ADA 2026 guideline



### Standards of Diabetes Care

- Comprehensive medical evaluation(PS61) – at diagnosis and at each follow- up
- Assessment of Comorbidities
  - Dental referral for comprehensive exam(annually) – grade E level of evidence PS69)
- (Under section 5) DSMES Referral (A), Medical Nutrition(A), Physical Activity(B), Smoking cessation (A), Behavioral Strategies(A), Psychosocial Care(A), Sleep Health(A)
- Annual Obesity screening using BMI
- A1C q3m (uncontrolled), q6m others
- Statin for  $\geq 40$  y/o; lipids annually or PRN
- UACR & BMP annually; ACEi or ARB w/ UACR>30
- Annual comprehensive foot exam
- Annual dilated eye exam
- Aspirin per ADA/USPSTF guidelines

# Standards of Care Contd..

## ❑ Recommended Vaccines – Page S67-S68

- ✓ **COVID-19** – Grade 3 level of evidence
- ✓ Hepatitis B <60yrol– Grade 1 level of evidence
- ✓ Influenza Annually (Grade 3 level of evidence)
- ✓ **RSV**: adults  $\geq 60$  years (Grade 1 level of evidence)
- ✓ **Tdap booster** q10yrs, extra dose(pregnant) (Grade 2 level of evidence)
- ✓ Pneumonia PPSV 13, PCV20 or PCV 15—(Grade 2 level of evidence)
  - ✓ Generally, PPSV23: 19-64 y/o (1 dose);  $\geq 65$  y/o, 2nd dose  $\geq 1$  year after first doce
  - ✓ **PCV20 or PCV 15**:  $\geq 65$  y/o & immunocompromised

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# Other potential Edits

- Indicating the Grade of Evidence /level of evidence
- E.g : A,B, C, E; **A(1)- Clear evidence(RCT's), B(2)- Supportive Evidence from Cohort), C(3)- Supportive Evidence from poorly controlled studies), E(4)- Expert consensus**